actuals basis. A vague notion of incentivizing the performance of primary healthcare facilities has also been expressed.

Interventions for Expanding Supply of Doctors and Paramedics Unspecific and Inadequate

While addressing the most critical constraint in expanding healthcare facilities in the country—rapidly increasing the supply of well-trained doctors and paramedics—the Policy relies only on generalities. ‘Strengthening existing medical colleges and converting district hospitals to new medical colleges to increase number of doctors and specialists’ has been ‘recommended’. It drifts when it only seeks to recognize ‘the need to increase the number of postgraduate seats’, or when it supports ‘expanding the number of AIIMS-like centres’ or when it ‘advocates a common entrance exam for UG entrance at the all-India level’ or when it ‘recognizes the need to improve regulation and quality management of nursing education’.

Similar ambiguity is on display in the case of paramedical skills. ‘Training courses and curriculum for super speciality paramedical care would be developed,’ it says. ‘The Policy recognizes the role played by physiotherapists, occupational and allied health professionals keeping in view the demographic and disease transition the country is faced with and also recognizes the need to address their shortfall’.

It also talks about the creation of a ‘Public Health Management Cadre’ in all states based on public health or related disciplines, as an entry criterion. But no rationale has been provided for this proposal.

Private Sector Largely Ignored

The policy only ‘suggests’ exploring collaboration for primary care services with ‘not-for-profit’ organizations with a track record of public service